BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal durations. The week-12 echo is relative to first HER2 treatment, not the first Phesgo injection after a switch. Booking and result review remain the treating team's responsibility.

Check	Before starting	Cycles 1–4	Around cycle 5 / week 12	Continuing treatment	Transition / follow-up
Blood tests	full blood count (FBC), renal and liver profile	FBC, renal and liver profile before every cycle. Review required counts before Day 8 and Day 15 dosing under the local pre-dose pathway.	Continue pre-cycle bloods	FBC, renal and liver profile before every cycle. Review required counts before Day 8 and Day 15 dosing under the local pre-dose pathway.	Review tests for the next active medicine
Echo / multigated acquisition (heart pumping) scan (MUGA)	Before the first HER2 treatment	Earlier if symptoms or concern	Echo due at about 12 weeks (cycle 5 if on time)	Every 12 weeks; 4–8 weeks if impairment	Keep the clock on maintenance
electrocardiogram (ECG) / cardiac markers	ESC: baseline ECG and cardiovascular risk review; baseline troponin/BNP if they will be used for monitoring	Repeat when symptoms, risk or the cardiac plan indicate	ECG does not replace echo	Markers are risk-based, not automatic tumour-marker tests	Follow up abnormal results
Scans / cancer markers	Baseline imaging of known disease	Clinical response; earlier imaging for concern	Response review generally every 2–4 months	Repeat appropriate imaging; markers only as adjuncts	Review benefit and next-line plan
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	Assess endocrine indication and fracture risk	No routine cycle-by-cycle DEXA	Plan baseline DEXA if aromatase inhibitor (AI) / ovarian suppression indicated	Repeat according to risk, results and endocrine therapy	Continue the agreed bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00797 v4 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Phesgo product information

THE WHOLE TREATMENT JOURNEY

Phesgo + weekly paclitaxel: the whole pathway

HER2-positive metastatic / unresectable locally recurrent breast cancer

Phesgo on Day 1 and paclitaxel on Days 1, 8 and 15 of a 21-day cycle, for up to eight paclitaxel cycles. Phesgo may continue alone while beneficial and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Baseline / previous treatment	Current combination	Response / phase review	Continuing Phesgo	Change / follow-up
Cancer medicines NCCP 00797	Confirm HER2 status, indication and baseline checks	Phesgo every 21 days with taxane visits	Review benefit and whether chemotherapy ends	Phesgo alone while beneficial and tolerated	Review outcomes and next plan
Surgery METASTATIC	Only if individually indicated	Coordinate with systemic treatment	multidisciplinary team (MDT) review	No routine surgical phase	Site-specific plan
Radiotherapy METASTATIC	Assess symptom / site need	Timing and pauses agreed by both teams	Reassess need	No assumed routine concurrent schedule	Site-specific review
Blood tests NCCP 00797	Baseline FBC, renal / liver	FBC, renal and liver profile before every cycle. Review required counts before Day 8 and Day 15 dosing under the local pre-dose pathway.	Check before changing phase	Phesgo-only maintenance: FBC, renal and liver profile before each 21-day cycle	Active medicines / abnormal results determine follow-up
Echo / heart tests NCCP 00797 · ESC · SMPC	Baseline echo / MUGA; ECG risk review	Symptoms can prompt earlier assessment	12 weeks from first HER2 dose; around cycle 5	Continue every 12 weeks including maintenance	Agree follow-up when treatment ends
Imaging / markers METASTATIC · MARKERS	Baseline disease assessment	Review clinical response	Generally every 2–4 months	Adapt to symptoms and known sites	Discuss results and next steps
Endocrine / DEXA BONE · EARLY	Check hormone receptors and fracture risk	Discuss the later plan	Baseline DEXA when indicated	Endocrine therapy may overlap HER2 / radiotherapy (RT) if agreed	Bone and endocrine follow-up

What each check is for

1

Blood tests

FBC, renal and liver profile before every cycle. Review required counts before Day 8 and Day 15 dosing under the local pre-dose pathway.

Check treatment safety.

Ask / **act:** The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: NCCP 00797

2

Echo / MUGA

Echo or MUGA before starting and every 12 weeks during treatment, including maintenance. At an uninterrupted 21-day schedule, the first 12-week check is around cycle 5; count elapsed time from the first HER2 dose. A delay or route/phase change does not restart the clock. Cardiac concern may require 4–8-week checks or urgent earlier assessment.

Monitor heart pumping function.

Ask / **act:** The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: NCCP 00797

3

Cancer imaging

Baseline imaging records the disease sites. ESMO recommends response review generally every 2–4 months, adjusted for symptoms, disease pace, sites and treatment. computed tomography (CT) chest/abdomen/pelvis is used when appropriate; the team selects the actual scan and review date. Tumour markers may supplement assessment, not replace scans or determine a treatment change alone.

Assess disease response or surveillance.

Ask / **act:** The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: METASTATIC

4

Bone health

If hormone-receptor-positive disease leads to an aromatase inhibitor or ovarian suppression, assess fracture risk and baseline DEXA when indicated. Repeat according to results, risk and the endocrine plan; DEXA is not a scan at each Phesgo cycle.

Reduce fracture risk when endocrine therapy affects bone.

Ask / **act:** The treating team records the booking owner, due window, result reviewer and action if overdue.

Sources: BONE

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-21 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.15